

Weight Loss - Unintended - Female (Child)

Algorithm 1124 | Child (12-17 years) | Gender at birth: Female

Definition

- Unintentional weight loss assessment decomposed from NHS.UK's published when-to-get-help guidance.
- Child patient.
- Female.

Initial assessment

1. [OPEN_TEXT] What is the person's exact age and sex, are they pregnant/recently postpartum, and what dated measured weights, units, time course, and growth trend are available? This protocol starts at age 12; younger children require an approved pediatric route.
2. [OPEN_TEXT] Was weight loss truly unintended? Ask privately and age-appropriately about food restriction, purging, laxatives/diuretics, excessive exercise, body-image distress, food insecurity, neglect/coercion, pregnancy, medicines/substances, and self-harm/safeguarding risk.
3. [OPEN_TEXT] Ask about inability to eat/drink, repeated vomiting/diarrhoea, dehydration/reduced urine, fainting, confusion, chest symptoms, severe weakness, fever/night sweats, pain, bleeding, swallowing difficulty, persistent cough, excessive thirst/urination, abdominal symptoms, and other new illness.
4. [CAREGIVER_OBSERVATION] What change has a safe parent or caregiver observed in behaviour, alertness, eating or drinking, sleep, school or play, and normal activity?

Triage and redirect questions

1. Redirect → Eating Disorder Concerns

Is intentional food restriction, fear of weight gain, purging, or a suspected eating disorder the main concern?

Intentional restriction or eating-disorder behaviours require a dedicated eating-disorder and mental-health pathway.

1. Disposition level 100

Is there inability to keep fluids down, severe dehydration or very little urine, fainting/collapse, confusion, seizure, chest pain, abnormal breathing, severe weakness/inability to stand, major bleeding, severe abdominal pain, suspected diabetic crisis, dangerous eating-disorder behaviour, or immediate self-harm/safeguarding danger?

Unintended weight loss is usually gradual, but acute dehydration, metabolic illness, bleeding, severe malnutrition/electrolyte disturbance, medical instability, or mental-health/safeguarding danger can be time-critical.

1. Disposition level 70

With emergency features excluded, is measured weight continuing to fall without intent, is there another symptom or medicine change, or is the person aged 12-17, pregnant/postpartum, medically vulnerable, possibly restricting/purging, or affected by food insecurity or safeguarding concerns?

NHS.UK guidance: arrange clinical review through a Qatar pathway approved by governance (GOVERNANCE_REQUIRED) as soon as possible if there is weight loss and other symptoms - the earlier the cause is found, the sooner it can be treated; weight loss without trying should always be checked.

Advice

Qatar emergency response for acute instability

- Call Qatar emergency services on 999 and do not drive. Keep the person supervised and at rest. Do not force food or fluids, give supplements/electrolytes, or alter insulin/other medicines when vomiting, confused, drowsy, unsafe to swallow, or medically unstable. For immediate self-harm or safeguarding danger, follow the approved emergency safety and welfare-check pathway. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: collapse, confusion, seizure, or inability to stand; abnormal breathing, chest pain, severe dehydration, bleeding, or severe abdominal pain; unsafe eating-disorder behaviour or self-harm risk.

Prompt in-person unexplained-weight-loss assessment

- Arrange prompt in-person assessment through the Qatar route and timing approved by governance. Bring dated weights/growth records and a complete medicine/supplement history. Do not prescribe high-calorie supplements, electrolytes, vitamins, appetite stimulants, laxative changes, or refeeding targets without assessment; rapid refeeding can be unsafe in severe malnutrition. Adolescents require private age-appropriate eating-disorder and safeguarding screening with approved caregiver involvement.
- Call Back If: weight loss accelerates; fainting, vomiting, dehydration, weakness, chest symptoms, bleeding, fever/night sweats, excessive thirst/urination, or self-harm concern.

First aid

- Call Qatar emergency services on 999 and do not drive. Keep the person supervised and at rest. Do not force food or fluids, give supplements/electrolytes, or alter insulin/other medicines when vomiting, confused, drowsy, unsafe to swallow, or medically unstable. For immediate self-harm or safeguarding danger, follow the approved emergency safety and welfare-check pathway. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Unintentional weight loss", <https://www.nhs.uk/conditions/unintentional-weight-loss/> (page last reviewed 28 July 2025)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from the cited NHS.UK pages under the Open Government Licence v3.0; it is not licensed STCC content.

Question.DispositionLevel=null marks a See More Appropriate Guideline redirect and is not a disposition row. Local extensions, destination mappings, safeguarding workflows, and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: 35bcf8c0bd0122aa3176e150f95b1080929002c45e37cd4176114f8913a16262 | Status: BLOCKED_BY_RESEARCH_GAP